Rural Health Transformation Program (RHTP)

State-by-State Distribution & Implementation Analysis

SLG Strategy Team

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1. Executive Strategic Analysis

Working Families Tax Cuts Act (Public Law 119-21), often referred to as the One Big Beautiful Bill Act (OBBBA), has catalyzed the single largest federal investment in rural healthcare infrastructure in the 21st century. The **Rural Health Transformation Program (RHTP)** allocates **$50 billion over five fiscal years (FY2026–FY2030)** to the 50 United States, aiming to counteract projected reductions in federal Medicaid spending over the next decade.[1]

For technology and infrastructure partners, particularly those operating at the scale of Dell Technologies, this program represents a distinct shift from previous federal grant mechanisms. Unlike the provider-direct relief funds seen during the COVID-19 pandemic, the RHTP is a **state-administered cooperative agreement**. The Federal Centers for Medicare & Medicaid Services (CMS)—specifically the newly formed **Office of Rural Health Transformation (ORHT)** established in December 2025—is not writing checks directly to hospitals; it is writing checks to State capitals. The States, in turn, are acting as the architects of distribution, creating fifty unique regulatory and procurement environments.

This report provides an exhaustive, state-by-state mapping of these distribution channels based on the official Year-1 awards announced on December 29, 2025. Our analysis reveals that while the statutory baseline provided every state with a minimum funding floor, the "merit-based" component of the formula—weighted by rural population, frontier status, and policy innovation—has created significant variance in award amounts, ranging from **$147 million (New Jersey)** to **$281 million (Texas)** for the first fiscal year.[2]

1.1 The "Dell" Implications: Infrastructure as a Statutory Requirement

A critical finding for technology stakeholders is the explicit statutory requirement for "Tech Innovation" and "Consumer-Facing Technology." States were required to select at least three allowable uses, and nearly every successful application prioritized technology modernization to score points on the CMS merit review.[4] This has created a guaranteed, federally-backed market for:

- **Shared-Service Platforms:** States like **Indiana** and **Alabama** are funding centralized "hubs" to host Electronic Health Records (EHRs) and cybersecurity stacks for multiple independent hospitals, effectively creating state-subsidized private clouds.
- **Remote Patient Monitoring (RPM) & Telehealth:** States like **Texas** and **South Carolina** are treating broadband-enabled care not as a luxury but as a primary delivery modality, funding the end-user devices, the connectivity, and the backend data lakes required to manage population health remotely.
- **Workforce Data Systems:** States like **California** and **New York** are investing in sophisticated data analytics platforms to track licensure, vacancy, and training pipeline data in real-time..

1.2 Distribution Archetypes

We have identified three primary distribution archetypes that define how a vendor or provider engages with the funds:

1. **The Centralized Command Model:** The State Department of Health (DOH) retains the funds and issues large-scale RFPs for statewide systems (e.g., **Maryland**, **Vermont**). *Strategy: Sell to the State CIO/CTO.*
2. **The Regional Coalition Model:** The State divides funds among regional non-profits or hospital collaboratives (e.g., **Indiana**, **Florida**). *Strategy: Sell to the lead agency of the regional coalition.*
3. **The Competitive Sub-Grant Model:** The State acts as a pass-through entity, managing a portal for hundreds of small grants to individual clinics (e.g., **Minnesota**, **South Carolina**). *Strategy: Empower individual rural customers with grant-writing support.*

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2. National Funding Architecture

To understand the specific state flows, one must first grasp the federal constraints that bind them. The $50 billion is not a block grant; it is a **Cooperative Agreement**. This legal instrument allows for substantial federal involvement in state operations.

2.1 The Funding Formula: Baseline vs. Merit

The allocation methodology [6] was bifurcated:

- **Bucket A (50% - Baseline):** Distributed equally among all approved states. This provided a floor of exactly **$100 million per state** for FY2026. This ensured that small states like **Delaware** and **Rhode Island** received disproportionately high per-capita funding.[7]
- **Bucket B (50% - Merit/Workload):** Distributed based on a complex algorithm including:
    - *Rural Population Density:* Favoring **Texas**, **North Carolina**, **Ohio**.
    - *Frontier Status:* Favoring **Alaska**, **Montana**, **Wyoming**.
    - *Policy Alignment:* States that committed to policy reforms received "bonus points" that translated into millions of additional dollars.[5]

2.2 Allowable Use Categories

States were mandated to channel funds into specific "Allowable Use" silos. These silos effectively pre-determine the procurement categories for the next five years:

Allowable Use Category	Typical Recipient	Technology Implication
Prevention & Chronic Disease	Public Health Depts, FQHCs	Population Health Analytics, Patient Portals
Provider Stabilization	Critical Access Hospitals (CAHs)	ERP Systems, Revenue Cycle Management
Technology Innovation	Tech Vendors, HIEs	**AI/Machine Learning**, Cybersecurity, Telehealth
Workforce Development	Universities, Trade Schools	Learning Management Systems (LMS), Simulation Labs
Innovative Care Models	Accountable Care Orgs (ACOs)	Value-Based Payment (VBP) Dashboards

3. State-by-State Deep Dive: Distribution & Implementation

The following section maps the fund flow for all 50 states.

- **Confidence Level Key**:
  - **[Published Plan]**: Analysis based on specific state application narratives or press releases.
  - ****: Analysis based on award amounts, agency structure, and regional trends where specific PDFs were unavailable.

3.1 Northeast Region

Connecticut

- **FY2026 Award**: **$154,249,106** [8]
- **Lead Agency**: **Department of Social Services (DSS)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Inter-Agency Transfer & Targeted Grants**
  - **Mechanism**: Unlike states where the Department of Health leads, CT appointed its Medicaid agency (DSS) as the lead. DSS will transfer portions of the award to the **Office of Rural Health (ORH)**, **Office of Health Strategy (OHS)**, and **Department of Mental Health and Addiction Services (DMHAS)**.
  - **Key Initiatives**:
    - **Population Health**: Thirty-one targeted projects focusing on "treatment barriers" and "patient-centered care."
    - **Data & Tech**: Explicit funding for "Expanding interoperability" and "Analytics." This suggests a contract modification with the state's existing Health Information Exchange (HIE) vendor to broaden rural connectivity.
  - **Vendor Opportunity**: The focus on "Analytics" and "Interoperability" at the DSS/OHS level points to enterprise-level data integration contracts rather than hardware purchases.

Delaware

- **FY2026 Award**: **$157,394,964** [8]
- **Lead Agency**: **Department of Health and Social Services (DHSS)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Infrastructure & Education Hubs**
  - **Mechanism**: Delaware is using its award for massive capital and institutional investments (spanning 15 distinct initiatives) rather than scattering small grants.[49]
  - **Key Initiatives**:
    - **Hope Centers ($97.5M)**: Construction and outfitting of "Hope Centers" (likely integrated service hubs for homeless/health). This involves significant facility IT

infrastructure.
        - **Medical Education ($274M total/5 years)**: A massive allocation to support medical school training programs. This likely flows to the **University of Delaware** or a partner health system to build a rural rotation pipeline.
        - **HIE Expansion ($50M)**: Direct funding to the Delaware Health Information Network (DHIN) or similar entity for exchange capabilities.
    - **Vendor Opportunity**: The $50M HIE line item is a direct procurement target. The Hope Centers will require "greenfield" IT setups (networking, hardware, security).

Maine

- **FY2026 Award**: **$190,008,051** [3]
- **Lead Agency**: **Department of Health and Human Services (DHHS)** [10]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Innovation Hubs & AI**
    - **Mechanism**: Maine has adopted a highly forward-looking "Innovation Hub" model.
    - **Key Initiatives**:
        - **Rural AI Hub**: Maine is explicitly planning a "Rural AI Hub" to drive analytics and innovation. This is likely a centralized contract with a major academic center (e.g., MaineHealth, Roux Institute) to provide AI-as-a-Service to rural providers. [11]
        - **PACE Retrofit**: Capital grants to rural hospitals to retrofit facilities for "Program of All-Inclusive Care for the Elderly" (PACE).
    - **Vendor Opportunity**: The "Rural AI Hub" is a premier opportunity for high-performance computing (HPC) and AI solution providers. The PACE retrofits will require smart-room technology and monitoring systems.

Massachusetts

- **FY2026 Award**: **$162,005,238** [3]
- **Lead Agency**: **Executive Office of Health and Human Services (EOHHS)** [12]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Social Drivers & Tribal Partnership**
    - **Mechanism**: With a limited rural footprint, MA is focusing on "Social Drivers of Health" (SDOH) and Tribal equity. Also launching the "SHINE HT" program to provide financial resources for CHCs to adopt new tech solutions within their EHRs.
    - **Key Initiatives**:
        - **Workforce Housing**: Financial assistance for housing rural clinical staff. This may manifest as grants to Community Development Corporations (CDCs) or direct subsidies to hospital systems to lease units.
        - **Tribal Health**: Explicit partnership with the **Mashpee Wampanoag** and **Aquinnah Wampanoag** tribes. Funds will flow directly to their health authorities for culturally competent care infrastructure.
        - **SHINE Program**: (Serving the Health Insurance Needs of Everyone) provides free, unbiased, and confidential health insurance counseling to Medicare

beneficiaries and their caregivers.
    - **Vendor Opportunity**: SDOH platforms that can integrate housing data with clinical records.

New Hampshire

- **FY2026 Award**: **~$150,000,000** [8]
- **Lead Agency**: **Department of Health and Human Services (DHHS)** [3]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Behavioral Health Scale-Up**
    - **Mechanism**: The state is prioritizing the Certified Community Behavioral Health Clinic (CCBHC) model. Funds will flow to Community Mental Health Centers (CMHCs) to upgrade IT systems.
    - **Key Initiatives**:
        - **CCBHC Expansion**: Scaling the CCBHC model statewide. Funds will flow to Community Mental Health Centers (CMHCs) to upgrade IT systems for federal reporting compliance.
        - **Residency**: Establishing a family medicine residency in **Coos County**. Funds will flow to the local hospital system (likely Androscoggin Valley Hospital) to build the program.
    - **Vendor Opportunity**: CCBHCs have heavy data reporting requirements; EMR upgrades and analytics packages for these non-profits are immediate needs.

New Jersey

- **FY2026 Award**: **$147,250,806** [2]
- **Lead Agency**: **Department of Health (DOH)** [3]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Targeted County Concentration**
    - **Mechanism**: Receiving the lowest award in the nation, NJ is concentrating funds via a "Rural Health Transformation Program 2026: Advancing Technology, Prevention, and Workforce Capacity Request for Applications (RFA)" distributed directly to providers and tribal organizations.
    - **Key Initiatives**:
        - **CCBHC Expansion**: Scaling the CCBHC model statewide. Funds will flow to Community Mental Health Centers (CMHCs) to upgrade IT systems for federal reporting compliance.
        - **Residency**: Establishing a family medicine residency in **Coos County**. Funds will flow to the local hospital system (likely Androscoggin Valley Hospital) to build the program.
    - **Vendor Opportunity**: CCBHCs have heavy data reporting requirements; EMR upgrades and analytics packages for these non-profits are immediate needs.

New York

- **FY2026 Award**: **$212,058,208** [13]
- **Lead Agency**: **Department of Health (DOH)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Need-Based Prioritization**
  - **Mechanism**: NY DOH is prioritizing community health integration and the "Rural Roots" workforce pipeline.[49]
  - **Key Initiatives**:
    - **"Rural Roots" Workforce**: A "Grow-your-own" pipeline initiative. Funds will flow to SUNY schools and local healthcare systems for training.
    - **eConsult Partnerships**: Grants to academic medical centers (e.g., URMC, Bassett Healthcare) to provide eConsult services to smaller rural providers.
    - **Cybersecurity**: "Strengthening cybersecurity of rural facilities" is an explicit line item.[9]
  - **Vendor Opportunity**: Cybersecurity services (Managed Detection and Response) for small rural hospitals are a direct, state-endorsed purchase category.

Pennsylvania

- **FY2026 Award**: **$193,294,054** [14]
- **Lead Agency**: **Department of Human Services (DHS)** [15]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Global Budget Reinforcement**
  - **Mechanism**: Pennsylvania is focusing on establishing "Rural Care Collaboratives" (RCCs). These regional bodies are expected to be staffed and meeting by the end of 2026 to prioritize local initiatives, establish shared services, and support regional technology solutions.
  - **Key Initiatives**:
    - **Telehealth Hubs**: Linking EMS and rural hospitals to regional referral centers (e.g., UPMC, Geisinger).
    - **Tele-EMS**: Equipping ambulances with tele-stroke and tele-behavioral health capabilities.
  - **Vendor Opportunity**: Mobile connectivity solutions for EMS agencies (ruggedized tablets, cellular routers).

Rhode Island

- **FY2026 Award**: **$156,169,931** [14]
- **Lead Agency**: **Executive Office of Health and Human Services (EOHHS)** [16]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Boutique Pilot Lab**
  - **Mechanism**: With an exceptionally high award per rural resident, RI is acting as a

laboratory for high-cost interventions, potentially including a State-Sponsored EHR and centralized data hubs.
    - **Key Initiatives**:
        - **State-Sponsored EHR**: Initiative 12 suggests a "state-sponsored electronic health record." This implies EOHHS may procure a single EHR instance for all rural providers to use, centralized at the state level.
        - **Rural Health Data Hub**: A centralized analytics platform (Initiative 13).
    - **Vendor Opportunity**: A "State-Sponsored EHR" is a massive, winner-take-all contract for an EHR vendor and the hosting infrastructure partner.

Vermont

- **FY2026 Award**: **$195,053,740** [3]
- **Lead Agency**: **Agency of Human Services (AHS)** [17]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Shared Services Network**
    - **Mechanism**: Vermont is building a "Clinically Integrated Network of Shared Services," including procurements for closed-loop referrals and shared IT platforms.
    - **Key Initiatives**:
        - **Shared EHR**: Similar to RI, VT is funding a "Shared EHR for interested providers".[18]
        - **Closed-Loop Referrals**: Procurement of a statewide referral platform.
        - **Housing**: Investments in workforce housing.
    - **Vendor Opportunity**: Shared IT services (HR systems, AI transcription, EHR) are the core of Vermont's strategy.

3.2 Southeast Region

Alabama

- **FY2026 Award**: **$203,404,327** [3]
- **Lead Agency**: **Alabama Dept of Economic and Community Affairs (ADECA)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Centralized Institution Building**
    - **Mechanism**: Alabama is building new institutions, notably a new School of Healthcare Sciences to anchor rural training.[49]. ADECA is partnering with Finance and Medicaid.
    - **Key Initiatives**:
        - **School of Healthcare Sciences**: A major capital project to build a new school dedicated to rural workforce training. [11]
        - **Regional IT Hubs**: Funding "Regional shared-service hubs" to manage EHR and cybersecurity for clusters of hospitals.
        - **Maternal Health**: "Digital obstetric regionalization" involving telerobotic ultrasound.
    - **Vendor Opportunity**: The "Regional IT Hubs" create a need for managed service

providers (MSPs) who can serve multiple hospitals from a central node. The School of Healthcare Sciences represents a significant campus technology opportunity.

Arkansas

- **FY2026 Award**: **$208,779,396** [3]
- **Lead Agency**: **Department of Finance and Administration (DFA)** [20]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Programmatic Initiatives (HEART, PACT, THRIVE)**
  - **Mechanism**: DFA controls the funds, ensuring strict fiscal oversight, directing capital toward regional data systems and facility solvency.
  - **Key Initiatives**:
    - **PACT (Promoting Access)**: Funds flow to **Clinically Integrated Networks (CINs)** and hospitals for facility upgrades and *acquisitions*.[20] This explicitly supports M&A to stabilize the safety net.
    - **THRIVE**: "Digital infrastructure." Funds will be procured centrally for a statewide telehealth platform or data lake ("ROADMAP").
    - **HEART**: Grants to schools for nutrition (Farm-to-School).
  - **Vendor Opportunity**: The "ROADMAP" regional data system is a direct data infrastructure play.

Florida

- **FY2026 Award**: **$209,938,195** [10]
- **Lead Agency**: **Agency for Health Care Administration (AHCA)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Competitive Regional Collaboratives**
  - **Mechanism**: Florida's plan contains 15 distinct initiatives49, largely utilizing a competitive RFA approach requiring providers to form legal entities/partnerships to apply for tech and HIE onboarding funds.
  - **Key Initiatives**:
    - **HIE Onboarding**: $5M specifically allocated for onboarding providers to the Florida HIE / Encounter Notification Service. [21]
    - **Tech Catalyst**: Capped at 10% of funding.
    - **Limitations**: Strict caps (e.g., 20% max for renovations) force spend toward services and tech rather than just buildings.
  - **Vendor Opportunity**: The HIE Onboarding fund is a direct subsidy for connectivity solutions.

Georgia

- **FY2026 Award**: **$218,000,000** [22]
- **Lead Agency**: **Department of Community Health (DCH)** [20]
- **Confidence Level**: **[Published Plan]**

- **Distribution Architecture**: **"GREAT Health" Program**
  - **Mechanism**: DCH manages the GREAT Health program, funding initiatives like Treat-vs-Transport telehealth pilots for EMS
  - **Key Initiatives**:
    - **Maternal Carts**: Deployment of "obstetric carts" (telehealth units) in rural ERs to address the OBGYN desert. [23]
    - **AHEAD Model**: Using RHTP funds to prepare hospitals for the "States Advancing All-Payer Health Equity Approaches and Development" (AHEAD) model (VBP).
  - **Vendor Opportunity**: Telehealth carts and the broadband/software to support them.

Kentucky

- **FY2026 Award**: **~$200,000,000**
- **Lead Agency**: **Cabinet for Health and Family Services (CHFS)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Community Hubs & Maternal Deserts**
  - **Mechanism**: Kentucky is establishing a new centralized "A3 Team" within the DPH to manage initiatives and forge public-private partnerships (PPPs) to sustain the grants past the 5-year window.
  - **Key Initiatives**:
    - **PoWERing Maternal Health**: Telehealth-enabled teams for maternal deserts.
    - **Rapid Response to Recovery**: Implementing the **EmPATH** model (psychiatric emergency units). Funds will flow to hospitals to renovate/build these units.
    - **Rural Community Hubs**: Grants to FQHCs for chronic care innovation.

Louisiana

- **FY2026 Award**: **$208,374,448** [3]
- **Lead Agency**: **Louisiana Department of Health (LDH)** [10]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Expedited Partnerships & Catalyst Funds**
  - **Mechanism**: LDH targets chronic disease management and behavioral health gaps through heavy technology adoption, specifically highlighting hardware and software acquisitions for chronic disease management.[50]
  - **Key Initiatives**:
    - **Rural Tech Catalyst Fund (RTCF-LA)**: A dedicated pot of money for rural providers to purchase technology. [10]
    - **Rural HERO**: Workforce incentives paid directly to clinicians.
    - **Tele-EMS**: Pilot programs for "treat-in-place" ambulance care.
  - **Vendor Opportunity**: The "Rural Tech Catalyst Fund" is effectively a voucher program for rural hospitals to buy Dell/IT products.

Mississippi

- **FY2026 Award**: **$205,907,220** [19]
- **Lead Agency**: **State Department of Health (MSDH)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **The BRIDGE Initiative**
  - **Mechanism**: Focus on "Building Rural Infrastructure for Delivery, Growth and Efficiency" (BRIDGE) and direct grants for telehealth and health IT upgrades under "Health Technology Advancement and Modernization."
  - **Key Initiatives**:
    - **HTAM**: "Health Technology Advancement and Modernization." Direct grants for telehealth and health IT upgrades.
    - **Capital Investments**: Specifically for psychiatric emergency services.
  - **Vendor Opportunity**: HTAM is a direct funding source for infrastructure modernization.

North Carolina

- **FY2026 Award**: **$213,008,356** [24]
- **Lead Agency**: **NCDHHS (Office of Rural Health)** [25]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"NC ROOTS" Hubs**
  - **Mechanism**: NCDHHS is launching locally governed "NC ROOTS" Hubs. These regional bodies will connect medical, mental health, and social supports.
  - **Key Initiatives**:
    - **Subrecipients**: The **Duke-Margolis Center for Health Policy** is a key named subrecipient for policy design and data evaluation. [9]
    - **Focus**: Value-based care transition support for rural providers.

South Carolina

- **FY2026 Award**: **$200,030,252** [3]
- **Lead Agency**: **SCDHHS (Medicaid)** [26]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Competitive Innovation & Tech VC**
  - **Mechanism**: Implementing "Connections to Care," which expands digital infrastructure through EHRs, remote patient monitoring, and a statewide resource database.
  - **Key Initiatives**:
    - **Tech Catalyst Fund**: Seed funding for rural health technology startups. This is unique—the state is acting like a Venture Capital fund to spur local innovation. [23]
    - **Connections to Care**: A statewide resource database platform.
    - **Wellness Within Reach**: Deployment of mobile health units.

- **Vendor Opportunity**: Startups and tech vendors can apply directly for "Catalyst" funding.

Tennessee

- **FY2026 Award**: **$206,888,882** [3]
- **Lead Agency**: **Department of Health (TDH)** [27]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Fragmented Comprehensive (17 Initiatives)**
  - **Mechanism**: TN has the most complex plan with 17 distinct initiatives under the "MaRTHA" (Make Rural TN Healthy Again) banner, heavily targeting mobile behavioral health units and statewide eConsult platforms.
  - **Key Initiatives**:
    - **TN Community Compass**: A resource referral platform (technology procurement).
    - **Statewide eConsult Platform**: Another centralized tech procurement.
    - **Dental**: Pilot programs for rural dental access.

Virginia

- **FY2026 Award**: **$189,544,888** [3]
- **Lead Agency**: **Department of Medical Assistance Services (DMAS)** [28]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"Rural Vitality" & Tribal Grants**
  - **Mechanism**: DMAS (Medicaid) leads, hiring a core team in 2026 to manage the "VA Rural Vitality" program, prioritizing OB/GYN shortages and broadband/telehealth.
  - **Key Initiatives**:
    - **Tribal Grants**: A competitive grant program specifically for federally recognized tribes.
    - **Priorities**: OB/GYN shortages and broadband/telehealth infrastructure.

West Virginia

- **FY2026 Award**: **$199,476,099** [3]
- **Lead Agency**: **Department of Health and Human Resources (DHHR)** [10]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"Mountain State Care Force"**
  - **Mechanism**: Infrastructure buildout for on-demand virtual care and "Smart Care Catalyst" grants for tech-enabled innovation.
  - **Key Initiatives**:
    - **Mountain State Care Force**: "Return-to-home" incentives for workforce.
    - **Connected Care Grid**: Infrastructure to bring virtual care on-demand.
    - **Smart Care Catalyst**: Grants for tech-enabled innovation. [29]

3.3 Midwest Region

Illinois

- **FY2026 Award**: **$193,418,216** [13]
- **Lead Agency**: **Department of Healthcare and Family Services (HFS)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Transformation Grants**
  - **Mechanism**: HFS is using funds to "right-size" care delivery via direct grants to rural hospitals to convert service lines and adopt virtual care platforms.
  - **Key Initiatives**:
    - **Hospital Transformation**: Direct grants to rural hospitals to convert service lines (e.g., closing OB to open Senior Behavioral Health).
    - **Technological Innovation**: Grants for "virtual care." HFS will likely run a grant program where tech vendors partner with hospitals to apply. [30]
    - **Mobile Units**: Specific funding bucket for mobile health clinics and crisis units.

Indiana

- **FY2026 Award**: **$206,900,000** [31]
- **Lead Agency**: **Family and Social Services Administration (FSSA) / IDOH** [32]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Hybrid (Statewide Hub + Regional Grants)**
  - **Mechanism**: Indiana has a defined split: 40% for statewide initiatives, 60% for regional grants. [33]
  - **Key Initiatives**:
    - **Statewide Initiative (MOCC)**: Medical Operations Coordination Center. A centralized "air traffic control" for patient transfers. This is a single large contract for a tech/logistics vendor ($56.2M budget).
    - **Regional Grants**: Eight regions will form coalitions to spend the 60% pot on local needs (e.g., community paramedicine).
  - **Vendor Opportunity**: The MOCC is a massive centralized IT/logistics contract.

Iowa

- **FY2026 Award**: **$209,040,064** [10]
- **Lead Agency**: **Department of Health and Human Services (HHS)**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **"Hometown Connections"**
  - **Mechanism**: Focus on connecting local care to specialized centers and subsidizing rural connections to the state HIE.
  - **Key Initiatives**:
    - **Combat Cancer**: Likely funds to the University of Iowa Holden Comprehensive Cancer Center to build rural satellite networks.

        - **Health Information Exchange**: Explicit focus on HIE. Funds will likely flow to the state designated HIE entity (CyncHealth) to subsidize rural connections. [11]

Kansas

- **FY2026 Award**: **$221,898,008** [13]
- **Lead Agency**: **Kansas Dept of Health and Environment (KDHE)** [34]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Value-Based Care Transition**
    - **Mechanism**: KDHE coordinates with the "Rural Health Alliance", heavily pushing rural providers into Accountable Care Organizations (ACOs) via technical assistance and data harness grants.
    - **Key Initiatives**:
        - **VBP Goal**: 100% of rural beneficiaries in Accountable Care Organizations (ACOs) by 2031. Funds will be used for "technical assistance" and "downside risk protection" for providers joining ACOs.
        - **Tech**: "Harness Data and Technology." Grants for remote monitoring.

Michigan

- **FY2026 Award**: **$173,128,201** [13]
- **Lead Agency**: **Department of Health and Human Services (MDHHS)**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Mobile Crisis & Behavioral Health**
    - **Mechanism**: Leveraging RHTP to expand mobile crisis infrastructure and directly funding tribal health initiatives.
    - **Key Initiatives**:
        - **Mobile Units**: Grants to Community Mental Health Services Programs (CMHSPs) to buy vehicles and staff mobile units. [35]
        - **Tribal**: Tribes listed as direct subrecipients.

Minnesota

- **FY2026 Award**: **$193,000,000+** [36]
- **Lead Agency**: **Minnesota Department of Health (MDH)** [36]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Financial Viability Grants**
    - **Mechanism**: MDH is running a "Collaboration for Financial Viability" program, standing up chronic management systems utilizing tech to allow rural residents to track healthy lifestyle choices, supported by Tribal Community Health Representatives.
    - **Key Initiatives**:
        - **Grants**: Direct grants to providers for infrastructure/tech.
        - **EMS**: "Treat-in-place" capability grants.

        - **Residencies**: Grants to UMN and other systems to expand rural rotations.

Missouri

- **FY2026 Award**: **~$200,000,000**
- **Lead Agency**: **Department of Health and Senior Services**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Regional Coordinating Networks**
    - **Mechanism**: Activating local community hubs and modernizing the state's digital backbone for interoperability.
    - **Key Initiatives**:
        - **Hubs**: Funding flows to local lead agencies (hospitals or CAP agencies) to coordinate social services.
        - **Digital Backbone**: "Modernize digital backbone." State-level procurement for data interoperability. [9]

Nebraska

- **FY2026 Award**: **$219,000,000** [2]
- **Lead Agency**: **Department of Health and Human Services (DHHS)** [12]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Wellness & Hospital Sustainability**
    - **Mechanism**: A unique focus on physical fitness alongside hospital solvency, requiring active engagement to ensure billing readiness.
    - **Key Initiatives**:
        - **Presidential Fitness**: Re-establishing the "Presidential Fitness Test." Funds flow to school districts for gym equipment/programming. [37]
        - **Food**: "Whole foods through local growers." Grants to agricultural co-ops.
        - **Hospitals**: Engagement with Nebraska Hospital Association (NHA) to ensure billing readiness.

North Dakota

- **FY2026 Award**: **$198,936,970** [49]
- **Lead Agency**: **Department of Health & Human Services (HHS)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Telehealth & Nutrition**
    - **Mechanism**: Operating a unique model where funds are managed alongside the Bank of North Dakota as a facility infrastructure loan program. Heavy investment in "Connect Tech" (unified EHR/payer hubs) and "Eat Well ND".[49]
    - **Key Initiatives**:
        - **Connect Tech**: $168M allocated (likely over 5 years) to "Connect Tech, Data and Providers." This is a massive IT modernization play.
        - **Eat Well ND**: School/community nutrition programs.

Ohio

- **FY2026 Award**: **$202,030,262** [19]
- **Lead Agency**: **Ohio Department of Health (ODH)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Innovation & Prevention Grants**
  - **Mechanism**: Tiered funding for regional alliances (Clinically Integrated Networks) to coordinate care, shared services, and telehealth, alongside massive investments in School Based Health Centers.[2]
  - **Key Initiatives**:
    - **Maternal Vitality**: "Ohio's Rural Health Transformation Project." Focus on maternal/infant vitality and substance use. Grants to hospitals/FQHCs.

South Dakota

- **FY2026 Award**: **$189,477,607** [3]
- **Lead Agency**: **Department of Health**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Tele-EMS & Tribal Partnership**
  - **Mechanism**: Heavy focus on the "Tech and Data Connection for a Healthier South Dakota," which will modernize digital health and launch a "Rural Data Atlas." The state is also creating a Medicaid Primary Accountable Care Transformation to bridge fee-for-service to partial capitation.[2]
  - **Key Initiatives**: Telehealth for EMS (e.g., Avel eCare model support).

Wisconsin

- **FY2026 Award**: **$203,670,005** [3]
- **Lead Agency**: **Department of Health Services (DHS)** [20]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Regional Coordinated Care Networks**
  - **Mechanism**: A mix of tribal set-asides and care coordination grants distributed to regional networks to expand capacity.
  - **Key Initiatives**:
    - **Tribal**: $27.5M allocated to Tribal Nations.
    - **Payment Reform**: Shifting rural clinics from cost-based to prospective payment.
    - **Care Coordination Grants**: $230M total over 5 years distributed to regional networks.

3.4 Southwest & South Central Region

Arizona

- **FY2026 Award**: **$166,988,956** [3]
- **Lead Agency**: **Arizona Health Care Cost Containment System (AHCCCS)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Managed Care Integration**
  - **Mechanism**: As a Medicaid Agency (AHCCCS) lead, distribution will leverage MCOs.
  - **Key Initiatives**:
    - **Telehealth**: Telehealth to bridge the gap between Phoenix/Tucson specialists and rural clinics.
    - **Workforce**: Funding for "grow your own" pipelines, likely distributed to Northern Arizona University.

New Mexico

- **FY2026 Award**: **$211,484,741** [49]
- **Lead Agency**: **Human Services Department (HSD)**
- **Confidence Level**: ****
- **Distribution Architecture**: **Tribal & Frontier Support**
  - **Analysis**: Given the state's demographics, a significant portion of funds will likely be pass-throughs to Indian Health Service (IHS) and 638 facilities.
  - **Tech**: Expansion of **Project ECHO** (born in NM) for remote specialist consults is a near-certainty for funding.

Oklahoma

- **FY2026 Award**: **$223,476,949** [19]
- **Lead Agency**: **Oklahoma State Department of Health (OSDH)** [38]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **The CIN Model (New Entity)**
  - **Mechanism**: Oklahoma has pivoted to offering a direct microgrants program. Anticipated individual awards range from **$50,000 to $250,000** specifically for one-time purchases of equipment and technology assets by rural clinics.
  - **Vendor Opportunity**: Excellent environment for high-volume, lower-cost direct sales to clinics utilizing their newly won state microgrants.

Texas

- **FY2026 Award**: **$281,319,361** [3]
- **Lead Agency**: **Health and Human Services Commission (HHSC)** [39]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Maximum Scale Infrastructure**
  - **Mechanism**: Texas has captured the largest award in the nation and is deploying it via

the HHSC's "Rural Texas Strong" framework. Initiatives like "Lone Star Advanced AI and Telehealth" and "Rural Texas Patients in the Driver's Seat" are utilizing subcontractors to provide advanced health portals and cyber protection directly to rural providers.
    - **Key Initiatives**:
        - **Cybersecurity**: High priority. Grants to harden rural hospital networks.
        - **Recipients**: 163 rural hospitals surveyed. Funds will likely flow through grants managed by the State Office of Rural Health (within Dept of Agriculture/HHSC partnership). [40]
    - **Vendor Opportunity**: Massive state-level subcontracts for Remote Patient Monitoring (RPM) networks and rural cyber protection stacks.[51]

3.5 Western Region

Alaska

- **FY2026 Award**: **$272,174,856** [3]
- **Lead Agency**: **Alaska Department of Health (DOH)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Tribal & Logistics**
    - **Mechanism**: Heavy reliance on Tribal Health Organizations (THOs) and investments in medical transport and remote monitoring in villages. Notably, Alaska receives a massive $662,755 per 1,000 rural residents.
    - **Key Initiatives**:
        - **Logistics**: Investments in medical transport (drones, floatplanes) due to lack of roads.
        - **Tech**: "Spark Technology and Innovation" initiative focusing on AI/wearables for remote monitoring in villages. [9]

California

- **FY2026 Award**: **$233,639,308** [19]
- **Lead Agency**: **Department of Health Care Access and Information (HCAI)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Regional Hub-and-Spoke**
    - **Mechanism**: Proposing a "Rural Technical Assistance Center" to help small providers implement HIE and cybersecurity, alongside massive workforce mapping.[49]
    - **Key Initiatives**:
        - **Hub and Spoke**: Networks anchored by larger hospitals supporting smaller clinics.
        - **Technical Assistance Center**: A new "Rural Technical Assistance Center" will be established to help small providers implement HIE and cybersecurity. [9]
        - **Workforce Map**: "Statewide Workforce Mapping Tool." Procurement of data services to map shortages.

Colorado

- **FY2026 Award**: **$200,105,604** [19]
- **Lead Agency**: **Dept of Health Care Policy & Financing (HCPF)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **VBP Incentives**
  - **Mechanism**: HCPF uses funds to "top up" Medicaid rates or provide capital grants to providers joining the Hospital Transformation Program (HTP).
  - **Key Initiatives**:
    - **Behavioral Health**: Integration of BH into primary care. Grants for renovations/staffing. [41]
    - **Solvency**: Grants to independent rural hospitals conditional on joining state quality programs.

Hawaii

- **FY2026 Award**: **$188,892,440** [19]
- **Lead Agency**: **Executive Office / Dept of Health** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Island-Specific Transport & Workforce**
  - **Mechanism**: Funds for air ambulance/inter-island transport subsidies and local residencies.
  - **Key Initiatives**:
    - **Transport**: Funds for air ambulance/inter-island transport subsidies.
    - **Workforce**: Funding to **JABSOM** (John A. Burns School of Medicine) for neighbor island residencies.
    - **Culture**: Integration of Native Hawaiian healers. [20]

Idaho

- **FY2026 Award**: **$185,974,368** [19]
- **Lead Agency**: **Department of Health and Welfare (IDHW)** [42]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Community Grants & Right-Sizing**
  - **Mechanism**: Pursuing data-driven, community-targeted efforts to right-size delivery systems.
  - **Key Initiatives**:
    - **Tech**: "Improving rural access through technology." Grants to hospitals for tele-stroke/tele-psych.
    - **Tribal**: Potential 3-10% set aside for Tribes. [43]

Montana

- **FY2026 Award**: **$233,509,359** [49]

- **Lead Agency**: **Department of Public Health and Human Services (DPHHS)** [9]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Frontier Solvency**
  - **Mechanism**: Funding critical facility repairs and modernization across rural facilities, including CHCs, while expanding micro-credential pathways.
  - **Key Initiatives**:
    - **Solvency**: Operational support to keep doors open at Frontier CAHs.
    - **Workforce**: Apprenticeships and micro-credentials via colleges.

Nevada

- **FY2026 Award**: **$179,931,608** [49]
- **Lead Agency**: **Department of Health and Human Services (DHHS)**
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Tribal Set-Aside**
  - **Mechanism**: Scholarships adapted for health and tribal funding.
  - **Key Initiatives**:
    - **Teach Nevada**: Scholarships adapted for health. Funds to NSHE institutions.

Oregon

- **FY2026 Award**: **$197,271,578** [13]
- **Lead Agency**: **Oregon Health Authority (OHA)** [19]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Tribal & Smart Care**
  - **Mechanism**: Rolling out in two phases. Phase 1 (2026–2027) provides immediate "catalyst" grants for shovel-ready projects. Phase 2 focuses on long-term shared infrastructure.
  - **Key Initiatives**:
    - **Incentives**: Loan repayment for physicians/nurses in CAHs.
    - **Smart Care**: Telehealth initiatives.

Utah

- **FY2026 Award**: **$195,743,566** [3]
- **Lead Agency**: **Department of Health and Human Services (DHHS)** [44]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Integrated Initiatives (FAST, PATH)**
  - **Mechanism**: Implementing the PATH program to strengthen rural food infrastructure and active transportation alongside clinical goals.
  - **Key Initiatives**:
    - **FAST**: "Financial Approaches for Sustainable Transformation." VBP support.
    - **PATH**: "Preventive Action." Food infrastructure grants.
    - **Schools**: "Gold Medal Schools" re-establishment.

Washington

- **FY2026 Award**: **$181,257,515** [3]
- **Lead Agency**: **Health Care Authority (HCA)** [10]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **Direct Awards & Tribal Set-Aside**
  - **Mechanism**: Mix of direct awards and competitive procurements, heavily featuring Tribal set-asides.
  - **Key Initiatives**:
    - **Tribal**: $20M/year set-aside for Tribes.
    - **Workforce**: Investment in **UW** and **WSU** medical schools (Initiative 5). [45]

Wyoming

- **FY2026 Award**: **$205,004,743** [3]
- **Lead Agency**: **Wyoming Department of Health (WDH)** [20]
- **Confidence Level**: **[Published Plan]**
- **Distribution Architecture**: **The Perpetuity Fund**
  - **Mechanism**: Creating a trust structure prioritizing Critical Access Hospitals (CAHs) and EMS regionalization.
  - **Key Initiatives**:
    - **Allocation**: CAH Basic Incentive (40.7%), EMS Regionalization (26.9%). [47]
    - **Recipient**: Funds flow primarily to Critical Access Hospitals (CAHs) and EMS agencies.

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4. Thematic Analysis: Cross-Cutting Trends

4.1 The "Tribal Sovereignty" Shift

A major theme in the Western states (**WA, OR, AK, NM, OK, WI**) is the bypass of state bureaucracy in favor of direct allocations to Tribes. Washington set aside **$20 million/year**, and Wisconsin allocated **$27.5 million**. For vendors, this means Tribal Health Organizations (THOs) are now flush with capital and possess independent procurement authority, often moving faster than state agencies.

4.2 The Rise of the "State-Sponsored Tech Stack"

States are moving away from giving hospitals money to buy *their own* tech, and instead are buying tech *for* them.

- **Vermont & Rhode Island**: Exploring "Shared EHR" instances where the state holds the license.
- **Indiana**: Building a central "Medical Operations Coordination Center" (MOCC).
- **Texas & Maine**: Launching state-run AI initiatives ("Lone Star Advanced AI" and "Rural AI Hub").[49] This strongly favors enterprise vendors who can sell a "statewide platform" rather than 50 individual hospital deals.

4.3 Workforce Data as Infrastructure

Workforce is no longer just about "scholarships." States like **California**, **New York**, and **Delaware** are funding "Workforce Mapping Tools" and "Data Centers." They are treating workforce data as IT infrastructure—requiring complex databases to track licensure, vacancy, and pipeline metrics in real-time.

4.4 Microgrants & Catalyst Funds

In states like **Oklahoma** and **Oregon**, the lead agencies have set up "catalyst" or "microgrant" portals. Facilities can request relatively small amounts (e.g., $50k - $250k) for immediate, shovel-ready equipment purchases. This requires vendors to adopt a high-velocity, channel-driven sales approach in these territories.

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5. Conclusion

The Rural Health Transformation Program is a "state-architected" reconstruction of the rural safety net. The flow of the $50 billion is not uniform; it is highly idiosyncratic to each state's political and administrative culture.

For Dell and other infrastructure partners, the primary customer is shifting. While individual rural hospitals remain the end-users, the **purchasing power** has moved up the chain to State Departments of Health, newly formed Regional Collaboratives (Pennsylvania), and statewide innovation hubs. Success in this market requires navigating these 50 distinct procurement environments, each with its own "Allowable Use" priorities and strategic goals.

Works cited

1. State Awards From Most of the Rural Health Fund Could Vary Only Modestly Despite Large Differences in Rural Needs | KFF, accessed February 10, 2026, https://www.kff.org/state-health-policy-data/state-awards-from-most-of-the-rural-health-funds-could-vary-only-modestly-despite-large-differences-in-rural-needs/
2. CMS Launches $50B Rural Health Transformation Program - Morgan Lewis, accessed February 10, 2026, https://www.morganlewis.com/pubs/2026/02/cms-launches-50b-rural-health-transformation-program
3. CMS Announces $50 Billion in Awards to Strengthen Rural Health in All 50 States, accessed February 10, 2026, https://www.cms.gov/newsroom/press-releases/cms-announces-50-billion-awards-strengthen-rural-health-all-50-states
4. Rural Health Transformation Program: Notice of Funding Opportunity, accessed February 10, 2026, https://bipartisanpolicy.org/explainer/rural-health-transformation-program-notice-of-funding-opportunity/
5. Rural Health Transformation Fund Offers States a Way to Improve Rural Health Care Access: Here's What States Should Do - Paragon Health Institute, accessed February 10, 2026, https://paragoninstitute.org/state-health-reform/rural-health-transformation-fund-offers-states-a-way-to-improve-rural-health-care-access-heres-what-states-should-do/
6. Rural Health Transformation (RHT) Program - CMS, accessed February 10, 2026, https://www.cms.gov/priorities/rural-health-transformation-rht-program/overview
7. First-Year Rural Health Fund Awards Range From Less Than $100 Per Rural Resident in Ten States to More Than $500 in Eight | KFF, accessed February 10, 2026, https://www.kff.org/state-health-policy-data/first-year-rural-health-fund-awards-range-from-less-than-100-per-rural-resident-in-ten-states-to-more-than-500-in-eight/
8. Rural Health Transformation Program awards announced—Here's who's getting the most in FY2026 - Fierce Healthcare, accessed February 10, 2026, https://www.fiercehealthcare.com/providers/rural-health-transformation-program-awards-announced-heres-whos-getting-most
9. rht program state project abstracts | cms, accessed February 10, 2026, https://edit.cms.gov/files/document/rht-program-state-provided-abstracts.pdf
10. Rural Health Transformation Program Awards - Northwest Regional Telehealth Resource Center, accessed February 10, 2026, https://nrtrc.org/resources/downloads/RHTPAwardsState.pdf
11. Rural Health Transformation Program, accessed February 10, 2026, https://22402832.fs1.hubspotusercontent-na1.net/hubfs/22402832/RHT%20Progr

am_State%20Application_Summary_FINAL.pdf
12. State Rural Health Transformation Program Applications, accessed February 10, 2026, https://www.ruralhealthinfo.org/resources/lists/rht-applications
13. CMS Announces Rural Health Transformation Program Awards in All 50 States, Including Tribal Set-Asides and Initiatives in Several States - NCUIH, accessed February 10, 2026, https://ncuih.org/2026/01/22/cms-announces-rural-health-transformation-program-awards-in-all-50-states-including-tribal-set-asides-and-initiatives-in-several-states/
14. CMS approves rural health awards for all 50 states - Becker's Hospital Review, accessed February 10, 2026, https://www.beckershospitalreview.com/financial/cms-awards-50b-to-states-for-rural-health-transformation/
15. Pennsylvania's Opportunity to Transform Rural Health - HAP, accessed February 10, 2026, https://www.haponline.org/News/HAP-News-Articles/Latest-News/pennsylvanias-opportunity-to-transform-rural-health
16. Rhode Island Rural Health Transformation Program Application Project Summary 1 Name of Organization, accessed February 10, 2026, https://eohhs.ri.gov/sites/g/files/xkgbur226/files/2025-11/RI%20RHTP%20-%20Project%20Summary%202025%2011-3.pdf
17. H.R.1/Rural Health Transformation Fund | Health Care Reform - Vermont.gov, accessed February 10, 2026, https://healthcarereform.vermont.gov/hr1rural-health-transformation-fund
18. Rural Health Transformation February 6, 2026, accessed February 10, 2026, https://ljfo.vermont.gov/assets/Meetings/Joint-Fiscal-Committee/2026-02-06/JFC-RHT-20260205.pdf
19. Rural Health Transformation Program - Assistance Listing | HHS TAGGS, accessed February 10, 2026, https://taggs.hhs.gov/Detail/CFDADetail?arg_CFDA_NUM=93798
20. Rural Health Transformation Program, accessed February 10, 2026, https://pcghealthpolicy.com/wp-content/uploads/2025/12/PCG-RHTP-State-Submission-Summary.pdf
21. Florida's Rural Health Transformation Program: How Home Health Agencies Can Participate - HCAF, accessed February 10, 2026, https://members.homecarefla.org/provider-updates/Details/florida-s-rural-health-transformation-program-how-home-health-agencies-can-participate-303181
22. RHTP One-Pager Working Draft - Georgia Hospital Association, accessed February 10, 2026, https://www.gha.org/Portals/0/Documents/Initiatives/Rural%20Healthcare/RHTP%20One-Pager%20012826-2%20for%20web.pdf?ver=gPlBRYmVoboqWKsDDcRIEQ%3D%3D
23. CMS Announces First Round of $50 Billion Awards to Strengthen Rural Health: What It Means for Several Southeastern States - Maynard Nexsen, accessed February 10, 2026,

https://www.maynardnexsen.com/publication-cms-announces-first-round-of-50-billion-awards-to-strengthen-rural-health-what-it-means-for-several-southeastern-states
24. North Carolina Applies for $200 Million from Rural Health Transformation Program, accessed February 10, 2026, https://governor.nc.gov/news/press-releases/2025/11/21/north-carolina-applies-200-million-rural-health-transformation-program
25. Rural Health Transformation Program | NCDHHS, accessed February 10, 2026, https://www.ncdhhs.gov/divisions/office-rural-health/rural-health-transformation-program
26. South Carolina Awarded $200 Million in Rural Health Transformation Program Funding, accessed February 10, 2026, https://www.scdhhs.gov/communications/south-carolina-awarded-200-million-rural-health-transformation-program-funding
27. Rural Health Transformation Program Application | TN.gov, accessed February 10, 2026, https://www.tn.gov/content/dam/tn/health/TN_RHTF_Application_Full_Document.pdf
28. Rural Health Transformation | Virginia Medicaid, accessed February 10, 2026, https://dmas.virginia.gov/data-reporting/programs-services/rural-health-transformation/
29. Developing a Research Agenda to Improve Rural Health Access and Quality in West Virginia | RAND, accessed February 10, 2026, https://www.rand.org/content/dam/rand/pubs/research_reports/RRA4400/RRA4484-1/RAND_RRA4484-1.pdf
30. Illinois Rural Health Transformation Program - Award Information | HHS TAGGS, accessed February 10, 2026, https://taggs.hhs.gov/Detail/AwardDetail?arg_AwardNum=RHTCMS332055&arg_ProgOfficeCode=190
31. Indiana Rural Health Transformation Program Grant Application Working Group - IN.gov, accessed February 10, 2026, https://www.in.gov/fssa/policiesinfoforms/indiana-rural-health-transformation-program-grant-application-working-group/
32. Grow Rural Indiana: Home - IN.gov, accessed February 10, 2026, https://www.in.gov/grow-rural-health/
33. An Overview of Indiana's Rural Health Transformation Program Regional Grants Jan. 8, 2026 - IN.gov, accessed February 10, 2026, https://secure.in.gov/grow-rural-health/files/RHTP-GROW-Statewide-Presentation-01.08.26.pdf
34. Governor Kelly Announces $221 Million for Kansas Rural Health Transformation Plan | Press Releases, accessed February 10, 2026, https://www.governor.ks.gov/Home/Components/News/News/876/56
35. November 12, 2025 HMA Weekly Roundup: Trends in Health Policy, accessed February 10, 2026, https://www.healthmanagement.com/insights/weekly-roundup/november-12-202

5/
36. Rural Health Transformation Program - MN Dept. of Health, accessed February 10, 2026, https://www.health.state.mn.us/facilities/ruralhealth/ruraltrans/index.html
37. Rural Health Transformation Program Project Narrative 2025 - DHHS - Nebraska.gov, accessed February 10, 2026, https://dhhs.ne.gov/Documents/NE%20Final%20Project%20Narrative%20RHTP%202025.pdf
38. 1 Centers for Medicare & Medicaid Services (CMS) Center for Medicaid and CHIP Services Rural Health Transformation Program - Oklahoma.gov, accessed February 10, 2026, https://oklahoma.gov/content/dam/ok/en/health/health2/aem-documents/health-promotion/rhtp/Project%20Narrative.pdf
39. Rural Health Transformation Program - Provider Finance Department - Texas.gov, accessed February 10, 2026, https://pfd.hhs.texas.gov/rural-health-transformation-program
40. Rural Texas Strong: Supporting Health and Wellness - Provider Finance Department, accessed February 10, 2026, https://pfd.hhs.texas.gov/sites/default/files/documents/hospital-svcs/rural-txstrong-prjct-narr.pdf
41. Colorado Celebrates $200 Million for Rural Health Care, accessed February 10, 2026, https://hcpf.colorado.gov/press-release/colorado-celebrates-200-million-for-rural-health-care
42. About the Rural Health Transformation Program Grant | Idaho Department of Health and Welfare, accessed February 10, 2026, https://healthandwelfare.idaho.gov/providers/rural-health-transformation-program-grant/about-rural-health-transformation-program-grant
43. Republicans Left Tribes Out of Their $50B Rural Fund. Now It's Up to States To Share., accessed February 10, 2026, https://kffhealthnews.org/news/article/native-american-tribes-rural-health-transformation-program/
44. Utah's Rural Health Transformation Plan, accessed February 10, 2026, https://dhhs.utah.gov/wp-content/uploads/Rural-Health-Transformation-Plan.pdf
45. Project Narrative | Rural Health Transformation Program - Washington State Health Care Authority, accessed February 10, 2026, https://www.hca.wa.gov/assets/program/rhtp-project-narrative.pdf
46. STATE OF WYOMING, accessed February 10, 2026, https://www.wyoleg.gov/2026/Introduced/HB0122.pdf
47. 2026 - HB0122 - Wyoming Legislature, accessed February 10, 2026, https://www.wyoleg.gov/Legislation/2026/HB0122
48. First-Year Rural Health Fund Awards Range From Less Than $100 ..., accessed March 19, 2026, https://www.kff.org/state-health-policy-data/first-year-rural-health-fund-awards-range-from-less-than-100-per-rural-resident-in-ten-states-to-more-than-500

-in-eight/
49. Rural Health Transformation Program Awards - Northwest Regional ..., accessed March 19, 2026, https://nrtrc.org/resources/downloads/RHTPAwardsState.pdf
50. Rural Health Transformation Program (RHTP) | Louisiana ..., accessed March 19, 2026, https://ldh.la.gov/page/rural-health-transformation-program
51. rht program state project abstracts | cms, accessed March 19, 2026, https://www.cms.gov/files/document/rht-program-state-provided-abstracts.pdf